

Axcel® METRONIDAZOLE - 200 TABLET

COMPOSITION:

Each tablet contains
Metronidazole 200mg

PRESENTATION:

White, biconvex, round tablet, with a single score line on one side and marked KOTRA logo on the other, 9.8mm in diameter.

INDICATIONS:

Treatment of protozoal infection, intestinal and extra-intestinal amoebiasis, lamblia, acute ulcerative gingivitis, urogenital trichomoniasis, giardiasis, and Vincent's infection.

PHARMACOLOGY:

Mechanism of Action:

Metronidazole is a systemic trichomonacide and an effective amoebicide. It is exceedingly active against a wide range of pathogenic micro-organisms, notably *Trichomonas vaginalis* and other species of trichomonas, *Entamoeba histolytica*, *Giardia lamblia*, *balantidium coli* and the causative organisms of acute ulcerative gingivitis.

Pharmacokinetics:

Metronidazole is usually well absorbed from the gastrointestinal tract after oral administration and widely distributed in body tissues. The biological half-life of Metronidazole is 6.2 hours after administration and the maximum concentrations occur in the serum after 1 to 2 hours and traces are detected after 24 hours. Pharmacologically, Metronidazole appears to be practically inert. Large doses in experimental animals affect neither the cardiovascular system nor respiration. Both unchanged metronidazole and several metabolites including an acid oxidation product and a glucuronide are excreted in the urine. Metronidazole diffuses across the placenta, and is found in the breast milk of nursing mothers in concentrations equivalent to those in the serum.

DOSAGE AND ADMINISTRATION:

To be taken orally.
Children under 12 years and infants should be treated with Axcel Metronidazole-200 Tablets.

For treatment of urogenital trichomoniasis:

For adults and children over 12 years: 200mg thrice daily for 7 days or 800mg in the morning

and 1200mg in the evening for 2 days (To prevent re-infection the consort should receive a course of treatment concurrently). Children 7 - 12 years: 100mg 3 times daily; 3 - 7 years: 100mg 2 times daily; 1 - 3 years: 50mg 3 times daily. The dosage of children aged 1 - 12 should be repeated for 7 days.

In amoebiasis of:

a) Invasive intestinal disease in susceptible subjects - For adults and children over 12 years: 800mg 3 times daily for 5 days or 2000mg once daily for 3 days. Children 7 - 12 years: 400mg 3 times daily; 3 - 7 years: 200mg 4 times daily; 1 - 3 years: 200mg 3 times daily. All dosage to be repeated for 5 days in children.

b) Intestinal disease in less susceptible subjects and "chronic amoebic hepatitis" - For adults and children over 12 years: 400mg 3 times daily for 5 days or 2000mg once daily for 2 days. Children 7 - 12 years: 200mg 3 times daily; 3 - 7 years: 100mg 4 times daily; 1 - 3 years: 100mg 3 times daily. All dosage to be repeated for 5 - 10 days in children.

c) Amoebic liver abscess and other forms of extra-intestinal amoebiasis - For adults and children over 12 years: 400mg 3 times daily for 5 days or 2000mg once daily for 2 days. Children 7 - 12 years: 200mg 3 times daily; 3 - 7 years: 100mg 4 times daily; 1 - 3 years: 100mg 3 times daily. All dosage to be repeated for 5 days in children.

d) Symptomless cyst passers - For adults and children over 12 years: 400-800mg 3 times daily. Children 7 - 12 years: 200-400mg 3 times daily; 3 - 7 years: 100-200mg 4 times daily; 1 - 3 years: 100-200mg 3 times daily. All dosage to be repeated for 5 - 10 days.

For Giardiasis:

For adults and children over 12 years: 2000mg. Children 7 - 12 years: 1000mg; 3 - 7 years: 600mg; 1 - 3 years: 400mg. All dosage to be given once daily for 3 days.

For acute ulcerative gingivitis or Vincent's disease:

For adults and children over 12 years: 200mg 3 times daily or 400mg 2 times daily. Children 7 - 12 years: 100mg 3 times daily; 3 - 7 years: 100mg 2 times daily; 1 - 3 years: 50mg 3 times daily. All dosage to be repeated for 3 days.

CONTRAINDICATION:

Metronidazole is contraindicated in patients with active disease of the CNS or with evidence or a history of blood dyscrasia. In the absence of microbiological diagnosis of the cause(s) of infection in a patient who is to be treated for urogenital trichomoniasis, there is possibility that an accompanying gonococcal infection might persist in a symptomless state after *Trichomonas vaginalis* has been eliminated.

WARNING AND PRECAUTION:

This product should not be used in patients with blood dyscrasia or with active disease of the CNS. Its use should be avoided during pregnancy. If its use is essential, the short high-dose regimens should not be used. Treatment should be discontinued promptly if ataxia or any other symptoms of CNS involvement occurs. Alcohol should not be taken in conjunction with the drug. Patients should be warned against driving or operating machinery when feeling drowsy. Patients with blood dyscrasias or with active disease of the

central nervous system should be used with great care of metronidazole. All patients receiving metronidazole for more than 10 days should be monitored and treatment discontinued if signs of peripheral neuropathy or CNS toxicity develop.

Cases of severe hepatotoxicity / acute hepatic failure, including cases with a fatal outcome with very rapid onset after treatment initiation in patients with Cockayne syndrome have been reported with products containing metronidazole for systemic use. In this population, metronidazole should therefore be used after careful benefit-risk assessment and only if no alternative treatment is available. Liver function tests must be performed just prior to the start of therapy, throughout and after end of treatment until liver function is within ranges, or until the baselines values are reached. If the liver function test become markedly elevated during treatment, the drug should be discontinued.

Patients with Cockayne syndrome should be advised to immediately report any symptoms of potential liver injury to their physician and stop taking metronidazole.

Use in Pregnancy and Lactation:

Use of metronidazole should be avoided during pregnancy, especially the first trimester and especially high dose regimens. Women taking metronidazole should not breast feed their babies.

SIDE EFFECTS:

Gastrointestinal discomfort, in particular anorexia, nausea, diarrhoea, epigastric distress and abdominal cramping, headache, vomiting, coated tongue, glossitis, stomatitis, dryness of the mouth and unpleasant taste; skin rashes, and less frequently, vertigo, depression, insomnia, drowsiness, urethral discomfort and darkening of the urine. Incoordination and ataxia are rare. Occasionally, numbness or paresthesia of an extremity and a temporary decrease in the total white-cell count may occur. Urticaria, flushing, pruritus, dysuria, cystitis, a sense of pelvic pressure, and dryness of the vagina or vulva have been reported following administration of the drug. In some individuals the consumption of alcohol with Metronidazole may produce a disulfiram-like effect.

DRUG INTERACTIONS:

When given in conjunction with alcohol, metronidazole may provoke a disulfiram-like reaction in some individuals. Acute psychoses or confusion have been associated with the concomitant use of metronidazole and disulfiram. Metronidazole enhances the anticoagulant effect of warfarin and may impair the clearance of phenytoin and of lithium. Plasma concentrations of metronidazole are decreased by the concomitant administration of phenobarbitone, with a consequent reduction in the effectiveness of metronidazole. Cimetidine has increased plasma concentrations of metronidazole and might increase the risk of neurological side-effects.

OVERDOSAGE AND TREATMENT:

Symptoms of overdose are as in side effects. Treatment is symptomatic and there is no specific treatment for gross overdose of the drug. Uneventful recovery has followed

attempts at suicide with quantities of 6000mg and 12000mg of metronidazole.

STORAGE:

Keep container well closed. Store below 30°C. Protect from light.

KEEP OUT OF REACH OF CHILDREN JAUHI DARI KANAK-KANAK

PACK QUANTITIES:

Available in blister pack of 10 x 10's and 100 x 10's.

Further information can be obtained from pharmacist, physician or the manufacturer.

MAL20032263AZ

Manufacturer and Product Registration Holder:

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